Questionnaire information · English v16

UNREVIEWED DRAFT · 05.11.2026.

Translation draft: not independently language- or clinical-reviewed. Not a validated diagnostic tool.

Purpose

One offline HTML file for adults (18+): 712 questions in 48 topic sections. It helps to gather information before a visit to a psychiatrist, psychologist, psychotherapist or other mental health professional. The questionnaire is not a diagnostic test, does not calculate a total score or risk level, and is not a substitute for professional evaluation. The range of questions does not guarantee coverage of all possible problems.

Opening and completing the questionnaire

1. Download 172_Questionnaire_EN_v16_DRAFT_NOT_RELEASED.html and open in a compatible browser. JavaScript may not work in Drive or ChatGPT preview. You need a browser that allows JavaScript execution in a local HTML file.
2. Press “Start/Continue”. First, you will see some introductory information, followed by your current safety questions and the reason for your visit. Your first and last name are optional.
3. For safety questions, select an answer to continue; “I don’t want to answer” is a valid choice. There is no need to provide a forced “Yes” or “No.” You can stop and open unfinished results at any time.
4. After the initial block, select additional sections, all sections, or mark the sections indicated by the specialist in the numbered list (for example, 02, 03, 06, 08). You can also continue without additional sections. Initial answers are included regardless of the section selection.
5. The selection itself is not saved. Press “Save and continue.” Additional questions and sections can be skipped. If you are changing an earlier answer, review the related answers.
6. Download the JSON continuation file before the break. It contains only the confirmed responses. Use a separate session for each person.

Navigation and section progress

The question and answer options are displayed first, followed by “Save and Continue.” The remaining controls are located below. The empty section jump list has been replaced with “Select sections / view progress.” Section selection is also available during the opening questions; selection does not skip the opening safety answers.

The list shows all 48 sections and their current check boxes. Gray means not started, yellow means started, green means completed; blue border and caption indicate the current section. Example: 40/40, Omitted: 5. Intentional omissions, refusals, and uncertain responses are counted as processed questions, but their count is shown separately. Conditionally hidden questions are not included in the denominator; therefore, the count may change as previous answers are changed. Green indicates completion, not health or safety. Answers for removed sections are retained in the private continuation file.

At the bottom of the page there are three separate expandable information blocks: usage/privacy; legal information/project/sources; people and organizations that helped with the project. The inappropriate VUGD link has been replaced with a simple indication to call 112. In the public statement, the technical CUSTOM14 label has been replaced with a clear explanation, separating professional review from scientific validation. Question codes and v15 data schema have been preserved; the interface version is v16.

A topic selected in the topic selection helper immediately selects related sections. Manual selections and sections of other marked topics are preserved when removing a topic. The user may also remove a suggested section. The selection takes effect after “Save section selection and continue”.

For regular questions, use “Skip question”; for initial safety questions, the “I do not want to answer” option is retained. Previously saved refusals are not overwritten by the system.

Report for the specialist

Open the results and “Prepare report for specialist”. The report starts with a compact overview of the visit: person, filler, reason “why now”, up to three priorities, current safety responses and family/care topic indications. The safety table contains exact retention times for responses. Topic abbreviations are not conclusions; the exact questions are in the appendix.

The full appendix is on by default. It includes question wording, answers, question time periods, retention times, and missing answer statuses. It starts on a new print page. Extensive answers or browser print settings may also extend the initial report; content is not truncated to artificially fit on a single page. The short report should not be considered a substitute for all answers.

The report can be copied, downloaded in HTML format, or printed/saved as PDF. Before sending, check the preview, select A4, and turn off browser headers/footers if necessary. The recipient's application may change the layout of the copied text.

A completed questionnaire does not mean that the doctor can no longer or should not clarify the safety questions. The current situation is changing. The export date does not update old answers. The “Review initial safety answers” button allows you to confirm them again. Refusal, uncertainty and unanswered are not “No”, but the answer “No” is not a confirmation of safety either.

Family and caregiving questions

Care also includes a child without illness or disability. Individual questions address overload, support, unwanted thoughts, desire to harm, attitude towards the specific thought, current intention/preparation, previous actions and reasons for not providing care. Fear of the thought is not the same as the intention to carry it out. Insufficient resources or health difficulties do not automatically equate to deliberate neglect.

Questions use specific actions and time periods, rather than asking for an admission of “I am violent.” There is no hidden lie detection, moral character assessment, or automatic “dangerous/safe” labeling. Responses with indicated prior actions remain separate from current safety questions.

Some clarifications only appear after the relevant answer. Refusal may hide a sensitive branch. Current intention, preparation, and child/caregiver safety are not dependent on previously acknowledged overload or unwanted thoughts. No algorithm can guarantee honest answers or detect all threats.

AI export and response status

A single downloadable TXT file contains the question ID, exact text, answer codes/texts, time periods, retention times, and answer status. The opening questions and the need to review imported safety answers are marked separately. A section may not be selected, but its opening questions are still included; the rest of the content remains unselected.

Not selected, conditionally hidden, unanswered, omitted, refused and unknown are not negative responses. AI can make mistakes; it does not replace a specialist. First/Last name is replaced in AI export, but identifiable information may remain in the free text of the medicine. Use of AI is voluntary.

Importing answers from earlier versions

v16 retains v15 question schema and accepts v15/v15.1 continuation files. The v14 answer migration by question ID is preserved. The v14 unchanged care questions M31_01 and M31_08 are no longer deleted unnecessarily. The content choices of the previous three priorities are combined into one question; the original entries remain in a private backup. Answers with different meanings from even older versions may require re-filling. Always keep the original JSON.

After the JSON load begins, security responses are prompted for review. The earliest time is retained until the response is revalidated. After a security review, the saved fill point is returned; browser reloads during review also preserve the returned point. Preserve the original JSON. No automatic search for earlier HTML files occurs.

When an additional section is removed, those responses remain in the private JSON but are not interpreted in the shared review; the exception is the independent start block.

Privacy and confidentiality boundaries

The survey works offline, does not automatically send responses, and does not track analytics. Approved responses may be saved in the browser and downloaded; they are not encrypted. Privacy cannot be guaranteed on a shared or monitored device. “New Survey” does not delete downloaded or already sent files.

Only provide your name if you plan to share an identifiable report with a professional. Complete sensitive questions in a safe, private place. Refusal is permitted. The questionnaire itself does not report to authorities; professional confidentiality and exceptions are determined by applicable regulations and law, including child protection obligations. Clarify these boundaries with the recipient; an unwanted thought does not in itself prove a crime.

No one automatically monitors the answers. In case of immediate danger, call 112; there is no need to complete the questionnaire or wait for an explanation from the AI.

Approaches used and independent review

The questionnaire also contains questions specifically developed for this project, rather than translations of validated scales. Before widespread distribution, it requires an assessment by a Latvian mental health specialist, a child/vulnerable persons safety specialist, and a Latvian language reviewer. It also needs to be tested for understandability among people with different education and experience.

- Approach to questions about violent behavior in mental health care — https://pmc.ncbi.nlm.nih.gov/articles/PMC11826930/
- Children's Rights Protection Law, including Article 73 — https://likumi.lv/ta/id/49096-bernu-tiesibu-aizsardzibas-likums

Verification status — English v16 DRAFT

Draft status: 712 translated question records and 48 section titles have been built from the unchanged Latvian v16 data model. JavaScript syntax and a basic automated Chromium startup were checked. This is **not** full functional, linguistic, clinical or Android verification. Existing LV tests do not certify English. Independent review of sensitive wording and authorized instrument translations is still required. Do not present this file as a professionally approved clinical test.